



## جوستاف روسي انترناشيونال - مصر (EPE) \* طلب علاج أول مرة / متردد

إسم المريض ربيعاً : هوaid محمد علي جادالله

العنوان : 28 عطفة الجامع من  
ش شيبان

المحافظة : القاهرة

رقم البطاقة : 26207090101201 رقم التليفون : 01004364379

العمر (تاريخ الميلاد) : 09-07-1962 المهنة : ربه منزل

### الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التامين الصحي-القوات المسلحة-الشرطة-  
الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة  
نفقات العلاج في حالة ثبوت غير ذلك  
الإحصائي الاجتماعي  
التوقيع: (-----) المقر بما فيه  
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام اشعه مقطعيه علي الصدر بالصبغه [10851] قسم العلاج : الاورام

التعليق على التشخيص : -----

### تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني  
الاسم: د/ندى حسن  
الاسم: د/نرمين رمضان  
الوظيفة: -----  
الوظيفة: -----  
الوظيفة: -----  
التوقيع: -----  
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التوقيع: -----  
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس  
وتلتزم جهة العلاج بلائحة المجالس الطبية.  
كما تقرر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.  
تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض  
بالمستشفى قبل الصرف.

### أعضاء اللجنة

أ.د. ----- /د. ----- /د. ----- /د. -----

### قرار اللجنة

التشخيص النهائي : -----  
خطة العلاج : -----  
المستشفى : -----  
جهة العلاج : -----  
ملاحظات : -----  
قسم : -----  
نفقات : -----

ختم  
شعار الجمهورية

مدير عام  
مديرية الشؤون الصحية

رئيس  
المجلس الطبي المتخصص

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|                                                                           |                                                                                     |
|---------------------------------------------------------------------------|-------------------------------------------------------------------------------------|
| <b>Name:</b> هرايد محمد علي جادالله<br><b>National ID:</b> 26207090101201 |  |
|---------------------------------------------------------------------------|-------------------------------------------------------------------------------------|

|           |
|-----------|
| تقرير طبي |
|-----------|

|                  |
|------------------|
| السيد            |
| تحية طبية وبعد,, |

|                       |                       |
|-----------------------|-----------------------|
| إيماء للطلب المقدم من | بشأن تقرير طبي للمريض |
| بقسم:                 | رقم طبي:              |

|                                          |
|------------------------------------------|
| نتشرف بان نوضح فيما يلي تقريرنا عن حالته |
|------------------------------------------|

|                                             |                                             |
|---------------------------------------------|---------------------------------------------|
| تاريخ وساعة الدخول :<br>-- --:-- yyyy-mm-dd | تاريخ وساعة الخروج :<br>PM 01:17 05-07-2026 |
|---------------------------------------------|---------------------------------------------|

|                                        |
|----------------------------------------|
| وبعد الكشف والفحوصات والمتابعة وجد انه |
|----------------------------------------|

|                                                                             |
|-----------------------------------------------------------------------------|
| المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Contrast-enhanced chest CT |
|-----------------------------------------------------------------------------|

|                         |                                   |
|-------------------------|-----------------------------------|
| التاريخ :<br>05-07-2026 | الوقت :<br>-- --:--               |
|                         | الطبيب المعالج :<br>د/شيماء لاشين |



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٧٨١٩



|                                         |                                                |
|-----------------------------------------|------------------------------------------------|
| Patient Name : الأستاذ / هويدا محمد علي | Request No. : 13006587                         |
| Patient No. : 26207090101201            | Request Date : 24/05/2026                      |
| Path No. : 1662/26                      | Verification Date : 01/06/2026                 |
| Sex : Female                            | Financial Category : نفقة الدولة - جوسفان رومي |
| Age : 63 Y                              | Department : Out Patient                       |
| Referreing Doc :                        | Registered by : .marwa mohamed youss           |

### Pathology Report

Clinical Data: Left breast mass in a patient with known Right breast cancer

Nature of Specimen: Tru-cut biopsy, collected on 24/5/2026, at 1:10 am, Cold ischemia time: 20 seconds.

#### Gross Examination:

-Fragmented grey white tissue cores, ranging in size between 0.5 cm & 1.2 cm long, totally submitted.

#### Microscopic Examination:

Examination reveals proliferative fibrocystic changes with usual ductal hyperplasia and columnar cell change. No atypia and no malignancy

#### Diagnosis:

Left breast mass, Tru-cut biopsy:

Proliferative fibrocystic changes with usual ductal hyperplasia and columnar cell change.  
No atypia and no malignancy

#### Signature

Prof. Dr. Maha Guimei

Verified by :

Dr Maha Guimie

Printing Date : 07/06/2026 14:42:47

Page 1 of 1

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

|                 |                            |                    |                             |
|-----------------|----------------------------|--------------------|-----------------------------|
| Patient Name    | : الأستاذ / هويدا محمد علي | Request No.        | : 13006043                  |
| Patient No.     | : 26207090101201           | Request Date       | : 22/04/2026                |
| Path No.        | : 1130/26                  | Verification Date  | : 09/05/2026                |
| Sex             | : Female                   | Financial Category | : جوستاف رومي - نفقة الدولة |
| Age             | : 63 Y                     | Department         | : Out Patient               |
| Referring Doc : |                            | Registered by      | : ,marwa mohamed youss      |

### Pathology Report

Clinical Data: Right breast 12 O'clock mass, "BIRADs 4c".

Nature of Specimen: Tru-cut biopsy, collected on 22/4/2026, at 1:10 am, Cold ischemia time: 20 seconds.

#### Gross Examination:

-Fragmented grey white tissue cores, ranging in size between 0.5 cm & 1.0 cm long, totally submitted.

#### Microscopic Examination:

- \* Histological type: Invasive lobular carcinoma.
- \* Grade 2 (differentiation 3, nuclear pleomorphism 2, mitosis 1).
- \* Lobular carcinoma in situ: Present.
- \* Ductal carcinoma in situ: Absent.
- \* Perineural invasion: Absent.
- \* Angioinvasion: Absent.
- \* Microcalcification: Absent.
- \* Tumor infiltrating lymphocytes (TILs): Mild.

#### Diagnosis:

Right breast mass, Tru-cut biopsy:

- Invasive lobular carcinoma, Grade 2
- For further IHC (ER, PR, Her2 & Ki67).

#### Signature

Dr. Walaa Ibrahim Ghoneim.

Verified by :

Dr Maha Guimie



Patient's Name: HOWAYDA MOHAMED ALI GAD ALLAH  
Date: 2026-03-31  
Referred by: Prof.Dr. CT NCAP+C  
Hospital: Goustave Roussey International Egypt

## **MULTISLICE POST CONTRAST CT SCAN OF THE NECK ,CHEST, ABDOMEN AND PELVIS:**

**TECHNIQUE:** Multislice axial CT images of the neck ,chest, abdomen and pelvis obtained from the lower brain to the symphysis pubis with multiplanar reconstruction. IV contrast was used.

### **FINDINGS:**

#### **Clinically reported right breast cancer**

##### **I- NECK:**

- Normal appearance of the naso-, oro- and hypopharynx with preserved parapharyngeal fat planes.
- Intact laryngeal skeleton with no glottic, supra or infra glottic masses.
- Normal course and calibre of the carotid and jugular vessels.
- Normal CT appearance of the submandibular and parotid salivary glands showing homogenous enhancement. No focal solid or cystic lesions.
- Few subscentemetric bilateral upper deep cervical and submandibular LNs are noted; likely non-specific , the largest at left level II measures 10x7 mm .

##### **(II) Chest:**

- Right breast mass 2.3 x 1.7 cm .
- Right enlarged axillary lymph nodes , the largest measures 4.2 x 2.1 cm .
- Few right lower lobe pulmonary nodules , the largest 6mm .

Patient's Name: HOWAYDA MOHAMED ALI GAD ALLAH  
Date: 2026-03-31  
Referred by: Prof.Dr. CT NCAP+C  
Hospital: Goustave Roussey International Egypt

- No CT evidence of hilar or mediastinal lymph node enlargement.
- No pleural or pericardial collections seen.
- No gross cardiac abnormality.

## (II) Abdomen and pelvis:

- The liver is average in size showing homogenous CT density , no focal lesions
- Patent and homogeneously enhancing portal vein.
- No intra or extra hepatic biliary radicles dilatation.
- Normal CT appearance of the pancreas, both kidneys, aorta and IVC.
- Normal CT appearance of the spleen .
- Regular filling of the urinary bladder showing no stones, masses .
- Multiple uterine myomas , the largest 8 x 6cm \*
- No retro-crural, or pelvic lymphadenopathy.
- No ascites.
- No worrisome lytic or sclerotic bony lesions .
- No vertebral structural collapse .

## OPINION:

- Right breast mass with axillary lymph nodes .
- Few pulmonary nodules likely non specific

Dr.Reham Khedr, MD

|                   |                                                    |
|-------------------|----------------------------------------------------|
| Patient ID: 94712 | Patient Name English: Howaida Mohamed Ali Gadallah |
| Age: ٦٣ سنة ٩ شهر | 2026/04/16 12:51:16 PM :Report Date                |

N 9V0

### **Contrast enhanced Spectral Mammography**

**CLINICAL SUMMARY:** 63 years old female patient with pathologically proven carcinomatous deposits of right supraclavicular lymph nodes of breast origin (+ve ER, +ve PR and -ve HER2)

**CC & MLO low energy & recombined (post contrast) images**

#### **FINDINGS:**

Background Uptake: marked nodular background enhancement.

ACR: C (heterogenous dense)

#### **Right breast:**

▪ **Low energy (Non contrast) images:** Right breast shows middle 1/3 upper central irregular shaped speculated dense lesion associated with architectural distortion.

▪ **Recombined (Post contrast) images:**

○ Right breast shows focal malignant process inform of small upper central middle 1/3 irregular spiculated heterogenous moderate mass enhancement and measuring about 6.5 x 6 mm and about 7 cm from the nipple and about 10 mm from the skin (**Index lesion**).

○ **Multiple small innumerable similar satellites** are noted at the anterior and middle 1/3s and to lesser extent posterior 1/3 scattered over whole breast and collectively measuring overall about 9.4 x 9.2 x 13.0 cm at its AP, TS and CC diameters and some of them at the retroareolar region and reaching nipple.

▪ **Complementary US:**

○ The right breast shows small irregular shaped spiculated hypoechoic lesion located at 12:00 clock position

○ Multiple small similar innumerable satellites are noted medial and lateral as well as inferior to index lesion.

○ Multiple enlarged axillary lymph nodes at level I, II and III as well as supraclavicular lymph nodes with effaced hila, one of them shows irregular margins at level I likely extracapsular extension and measuring about 21 x 15.6 mm.

#### **Left breast:**

▪ **Low energy (Non contrast) images:** Left UOQ focal asymmetry.

▪ **Recombined (Post contrast) images:** Small UOQ middle 1/3 faint heterogeneous mass enhancement and measures about 16 x 12 mm and about 10 cm from the nipple.

▪ Normal skin / nipple / areolar complex.

▪ **Complementary ultrasound:**

○ Small ill-defined irregular hypoechoic mass noted at 12:00 clock position measuring about 8.4 x 4.8 mm and about 9.7 mm from the skin.

○ Multiple enlarged axillary lymph nodes with preserved oval shaped, one of them shows diffuse cortical thickening measuring about 18.3 x 11.2 mm with cortical thickening about 3.6 mm.

#### **IMPRESSION:**

• **RIGHT BREAST:** (BIRADS 6= pathological proven) Right multiple scattered diffuse enhancing masses as described above.

• **LEFT BREAST:** (BIRADS 4) left small UOQ middle 1/3 faint heterogeneous mass enhancement for tru cut biopsy.

*Best regards,*  
**Dr. Hedaya Nour, MD**

|                                            |                                                |
|--------------------------------------------|------------------------------------------------|
| Patient Name : الأستاذة / هويدا محمد علي : | Request No. : 13006043                         |
| Patient No. : 26207090101201               | Request Date : 22/04/2026                      |
| Path No. : 1130/26                         | Verification Date : 09/05/2026                 |
| Sex : Female                               | Financial Category : جوستاف روسي - نفقة الدولة |
| Age : 63 Y                                 | Department : Out Patient                       |
| Referreing Doc. :                          | IHC No. :                                      |
|                                            | Registered by : .marwa mohamed yousse          |

**IMMUNOHISTOCHEMISTRY REPORT**

**Clinical Data:** Right breast 12 O'clock mass, "BIRADs 4c" diagnosed invasive lobular carcinoma grade with lobular carcinoma in situ.  
**Nature of Specimen:** Tru-cut biopsy, collected on 22/4/2026, at 1:10 pm, Cold ischemia time: 20 seconds.

**Technique:**  
 Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for; ER clone (1E2), PR clone (SP1), Her2 clone (4B5), Ki67 clone (30-9) & SISH.

**Results:**  
 IPT (All controls showed appropriate reactivity):  
 ER: Strong nuclear reaction in 100% of tumor cells (score 8/8).  
 PR: Strong nuclear reaction in 95% of tumor cells (score 8/8).  
 Her2: Intermediate complete membrane reaction in more than 10% of tumor cells (score 2+).  
 SISH: Not amplified.  
 Average Her2 copy No.: (2.75), Her2/CEP17 ratio: (1.57).  
 Ki67%: 5%.

**Diagnosis:**  
 Right breast mass, Tru-cut biopsy:  
 Invasive lobular carcinoma, Grade 2  
 PT:  
 ER: Positive (score 8/8).  
 PR: Positive (score 8/8).  
 Her2: Equivocal (score 2+).  
 SISH: Not amplified, Group V.  
 Average Her2 copy No.: (2.75), Her2/CEP17 ratio: (1.57).

Verified by :  
 Dr Maha Guimie



|                |                          |                    |                           |
|----------------|--------------------------|--------------------|---------------------------|
| Patient Name   | الأستاذ / هويدا محمد علي | Request No.        | 13006043                  |
| Patient No.    | 26207090101201           | Request Date       | 22/04/2026                |
| Path No.       | 1130/26                  | Verification Date  | 09/05/2026                |
| Sex            | Female                   | Financial Category | جوستاف رومي - نفقة الدولة |
| Age            | 63 Y                     | Department         | Out Patient               |
| Referring Doc. |                          | Registered by      | .marwa mohamed yousse     |

**IMMUNOHISTOCHEMISTRY REPORT**

Ki67%: 5%.

Scoring system: "ER, PR"  
 ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, 1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66%; and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.  
 Scoring system: "Her2"  
 Reference: ASCO-CAP Her2 Test Guideline Recommendations. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low): as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

ISH groups:  
 The ratio is calculated and the results is interpreted as:  
 Group I (Positive): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cell is more than 1.  
 Group II (Negative): HER2/CEP17 ratio is more than 2 & Average HER2 copy number/ cell is less than 1. (Recounting give the same result).  
 Group III (Positive): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is more than 1 (Recounting give the same result).  
 Group IV (Negative): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is more than 1 but less than 6. (Recounting give the same result).

Verified by :  
 Dr Maha Guimie



|                |                           |                    |                           |
|----------------|---------------------------|--------------------|---------------------------|
| Patient Name   | الأستاذة / هويدا محمد علي | Request No.        | 13006043                  |
| Patient No.    | 26207090101201            | Request Date       | 22/04/2026                |
| Path No.       | 1130/26                   | Verification Date  | 09/05/2026                |
| Sex            | Female                    | Financial Category | جوسثاف روسي - نفقة الدولة |
| Age            | 63 Y                      | Department         | Out Patient               |
| Referring Doc. |                           | Registered by      | .marwa mohamed yousse     |

**IMMUNOHISTOCHEMISTRY REPORT**

Group V (Negative): HER2/CEP17 ratio is less than 2 & Average HER2 copy number/ cell is less than

Signature  
Dr. Walaa Ebrahim Ghoneim.

Verified by :  
Dr Maha Guimie

Printing Date : 10/05/2026 14:00:40

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Name : Howaida Mohammed Ali Jadallah  
Branch : Dokki  
Ref.Physician Prof : Goustav Roussey Egypt  
Date : 10-09-2026 07:47  
Medical No. : 458521  
Ins.Company : تعافى مستشفيات

## NUCLEAR MEDICINE BONE SCAN REPORT

Clinical Indication: 64-year-old female with right breast carcinoma. Referred for routine follow-up skeletal evaluation.

Procedure: Whole-body planar bone scintigraphy, anterior and posterior views, following intravenous administration of 99mTc-MDP.

### Findings:

- A small faint focal area of increased tracer uptake is noted at the L4 vertebral body.
- A small focal area of increased uptake is also noted at the lower sternum.
- Mild diffuse age-related degenerative changes are seen throughout the spine.
- The remainder of the axial and appendicular skeleton shows symmetrical physiological tracer distribution without additional focal lesions.
- Renal excretion is prompt and symmetric. No abnormal soft tissue uptake.

### Impression:

- Small focal uptake at the lower sternum and L4 vertebral body—equivocal osseous lesions. Differential includes early/low-grade metastatic involvement versus benign etiology, for radiological correlation.
- Follow up imaging as clinically indicated.

Dr. Alaa Nagi

MD Nuclear Medicine